

CoverFight

AI-powered insurance appeal automation

Your insurance said no. We make them reconsider.

The system is designed for you to give up

- 45 million insurance claims denied every year in the US
- Only 0.1% of patients ever file a formal appeal
- Of those who do appeal, 40–68% win — depending on denial type
- Most patients don't appeal because they don't know how, don't have time, or assume it won't work
- Insurers count on this. Denials are a profit mechanism, not a clinical judgment.

\$300B

in denied claims
annually

The appeal letter is the entire lever

A formal written appeal citing applicable federal law — ERISA, the ACA, the No Surprises Act — forces insurers into a defined legal process. They must respond. They must assign a qualified reviewer. They cannot simply ignore it.

Most patients never send one.

We send one for every single case, automatically, within 24 hours.

From denial to filed appeal in under 3 minutes

01

Patient submits

Fill out one form. Paste the denial reason. Upload the letter if available. Done.

02

AI analyzes and writes

We classify the denial, identify the strongest legal arguments, and draft a formal appeal letter tailored to the specific case.

03

We file it

With authorization, we send the appeal via certified mail or fax directly to the insurer. Patient tracks status by email.

Two markets, one pipeline

Patients (B2C)

- Submit directly via coverfight.com
- No win, no fee — \$99 flat only when appeal is approved
- Card pre-authorized at submission, charged only on reversal
- Zero friction: no account, no lawyer, no paperwork

Medical Practices (B2B)

- Practices forward denied claims to us
- We handle everything — analysis, letter, filing, tracking
- Billed per case (\$150–300) or % of recovered revenue (5–10%)
- No new software for staff. We take it off their plate entirely.

Employer benefits (B2B2C)

Employers pay a per-employee-per-month (PEPM) fee to offer CoverFight as a workplace benefit — the same way they offer Teladoc or EAP programs. Employees access the patient flow directly. HR never touches a case. One contract covers thousands of potential users.

160M

Americans with employer-sponsored
health insurance

111%

Increase in average deductible over the
last decade

1 contract

Can cover 500 to 5,000 employees

Unit economics that work at every tier

Channel	Revenue per unit	Volume potential	Margin
B2C Patient	\$99 per win	Millions of annual denials	~85%
B2B Practice	\$150–300 per case	200+ cases/month per mid-size practice	~75%
B2B2C Employer	\$2–5 PEPM	1 contract = 500–5,000 employees	~90%

A single regional hospital system generates \$600K–\$1.2M in annual revenue at \$250/case

Three forces converging right now

1 AI maturity

Large language models can now read a denial letter, classify it, identify applicable law, and draft a compliant formal appeal — in seconds. This was not possible three years ago.

2 Regulatory tailwind

The ACA, ERISA, and the No Surprises Act (2022) have created a dense framework of patient rights that most patients don't know they have. We put those rights to work on their behalf.

3 Public anger

Insurance denial has become a mainstream political and cultural issue. Patients are looking for tools. Employers are looking for benefits. Providers are looking for relief.

What makes this defensible

Outcome data

Every case processed trains our classification and win-probability models. The more cases we handle, the better our predictions and strategies become.

Denial library

We build a proprietary database of denial language, insurer patterns, and winning appeal arguments. Impossible to replicate without volume.

Trust infrastructure

Certified mail filing, HIPAA-compliant handling, legal audit trails. This is table stakes that takes time to build properly.

Network effects

Employer and practice relationships compound. One hospital CFO referral unlocks 10 departments.

What we've built

- Fully functional AI pipeline: submission → classification → analysis → letter generation → filing
- 8 denial categories covered with category-specific legal strategies
- Admin dashboard for case management, AI chat, and external review generation
- Certified mail (Lob) and fax (Twilio) delivery integrations built
- Every fact in the appeal letter is traceable to patient-submitted data
- Currently in closed testing

How we grow

0–6 months

Prove it with patients

Drive B2C volume through SEO, social, and word of mouth. Build the case outcome dataset.

Target: 500 cases

6–18 months

Land practices

Sign 10 medical practices as managed service clients. One billing manager referral = practice-wide adoption.

Target: \$50K MRR

18–36 months

Employer channel

Partner with 3–5 benefits brokers who distribute to employer clients. One broker = hundreds of employer groups.

Target: \$500K MRR

What we're raising

\$750K

Seed Round

Looking for investors with healthcare, insurtech, or B2B SaaS experience who can open doors to hospital systems and benefits brokers.

Use of funds

- 40%** Engineering — payments, multi-tenant B2B portal, EHR integrations
- 30%** Sales — first B2B practice and employer contracts
- 20%** Legal & compliance — HIPAA, state regulatory review
- 10%** Operations

Most people never appeal.
The ones who do, often win.

We make sure everyone does.

[founder name] | [email] | coverfight.com